Spine Board



Category: **Medical, Healthcare & First Aid** Type: **Full SOP** Priority: **High** 3 labs

Covers these lab items: Spine Board

What it is used for

A spine board is a rigid patient-transfer device used by trained emergency responders when a patient requires specific movement or extrication precautions. Correct patient assessment, coordinated handling, secure positioning, and prompt transfer to appropriate medical care help reduce avoidable movement and further injury.

Main parts

- Rigid board supports the patient during transfer.
- Handholds allow responders to grip and manoeuvre the board.
- Patient straps secure the patient to the board.
- Head immobilisation blocks or supports help limit head movement where indicated.
- Head straps or forehead and chin restraints help secure the head where appropriate.
- Padding supports comfort and helps fill gaps where indicated.
- Compatible lifting or transfer accessories assist trained responders where approved.

Before you start

1. Only trained emergency or healthcare personnel should use the spine board.
2. Assess the scene for hazards and follow the applicable emergency response protocol.
3. Assess the patient's airway, breathing, circulation, and immediate life threats before moving when feasible.
4. Use a spine board only when indicated by the applicable protocol; it is not a routine long-duration support surface.
5. Inspect the board, straps, buckles, handholds, and head supports for damage or contamination.
6. Prepare enough trained responders to coordinate the move and explain the process to the patient when possible.
7. Wear appropriate PPE and prepare the destination stretcher or transfer surface before lifting.

How to use

1. Approach the patient using the approved scene-safety and patient-assessment procedure.
2. Determine whether a spine board is indicated and select the appropriate transfer method under the response protocol.
3. Assign clear roles and a coordinated command for each movement.
4. Maintain airway access and monitor breathing, circulation, and neurological status throughout the move.

5. Position the board using the approved technique while minimising unnecessary twisting or movement.
6. Transfer the patient onto the board with coordinated assistance and maintain alignment as clinically indicated.
7. Apply appropriate padding and secure straps without restricting breathing or circulation.
8. Use head supports only as indicated and ensure they do not obstruct airway assessment or access.
9. Recheck the patient's condition, strap security, comfort, and circulation after securing.
10. Lift and transfer the board with sufficient trained responders using coordinated commands.
11. Move the patient promptly to the prepared stretcher or appropriate care destination and follow handover procedures.
12. Clean and inspect the board and accessories after use, and document the transfer according to protocol.

⚠ Safety

- Do not move a patient on a spine board unless required for immediate safety or directed by the applicable clinical protocol.
- Airway, breathing, circulation, and other life-threatening conditions take priority over immobilisation.
- Do not leave the patient unattended or secured in a position that compromises breathing or circulation.
- Never lift the board by straps or accessories not designed as lifting points.
- Use enough trained responders to manage the patient and board safely.
- Follow local protocols for spinal motion restriction, patient monitoring, infection control, and equipment decontamination.

Care & storage

- Clean and disinfect the board, straps, and accessories using approved products after each use.
- Inspect the board for cracks, sharp edges, damaged handholds, and structural deformation.
- Check straps, buckles, and head supports for wear and reliable fastening.
- Store the board dry and in its designated location, ready for emergency use.
- Remove damaged equipment from service and arrange authorised replacement or repair.

Common mistakes to avoid

- Using a spine board for prolonged routine transport when an appropriate alternative is indicated.
- Moving the patient without coordinated commands or sufficient trained responders.
- Securing straps too tightly or obstructing airway access.
- Failing to reassess the patient after movement and securing.

Disclaimer: This SOP is prepared from general information available on the internet. For detailed instructions, refer to the manufacturer's original manual / file.